

SANITIZED SAMPLE REPORT

# CASH-MEDICAL LEAD LEAK REPORT

This sample uses an anonymized Atlanta-area cash-medical specimen. Real names, screenshots, phone numbers, patient details, and identifying marks are intentionally excluded. The point is to show how BADGR turns technical findings into decisions a practice owner can understand.

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## Report Snapshot

**38**

MOBILE PERFORMANCE

**82**

ACCESSIBILITY

**79**

BEST PRACTICES

**64**

SEO

### OWNER SUMMARY EXAMPLE

The site has the bones of a legitimate practice, but it feels slower and less guided than the kind of visitor it wants to attract. For a cash-pay patient, that matters. They are not just looking for information; they are deciding

whether this practice feels safe enough to contact and valuable enough to pay out of pocket.

## WHAT GETS CHECKED



### SPEED + FIRST IMPRESSION

Translate load delays into plain consequences: people wait, lose confidence, and leave before they understand the offer.



### MOBILE CALL + BOOKING PATH

Check whether a phone visitor can see the right service, trust the practice, and call or book without hunting.



### TRUST + COMPLIANCE-AWARE SIGNALS

Review policies, no-PHI language, service clarity, financing cues, testimonials, and proof that make a cash-pay patient feel safer.

## EXAMPLE PRIORITY NOTES

### PRIORITY 1

#### **Make the first screen simple enough to act on.**

The visitor should not have to decode the page. In cash medical, every extra second of confusion is like putting one more locked door between the patient and the front desk.

## PRIORITY 2

**Turn mobile calls and bookings into the obvious path.**

Most prospects are checking from a phone. If the call button, consult button, and service promise are scattered, the site is asking a busy person to do office work before they can become a lead.

## PRIORITY 3

**Add trust without asking for private details.**

A patient should see safety, professionalism, and next steps before a form asks for information. Forms should request business-safe contact details, not medical history or PHI.

 **CUSTOMER-FACING EMAIL DRAFT**

Your website is doing some things right, but it is making new visitors work too hard before they can trust you.

Think of the site like your front desk. If someone walks in and the lights are dim, the signs are confusing, and the intake clipboard is hard to find, they may leave even if your care is excellent. Online, slow loading, unclear buttons, and weak trust signals create that same feeling.

The first fix is not a giant rebuild. The first fix is to clean up the path: make the main service obvious, make mobile calling easy, reduce heavy page weight, and show the trust details a cash-pay patient needs before they reach out.

For this type of site, I would start with the 14-Day Cash-Medical Lead Leak Fix. If the audit proves the foundation is too weak, I will say so plainly before you pay for patchwork that will not hold.

**SAMPLE CORE SIGNALS**

|                            |       |
|----------------------------|-------|
| First visible content      | 3.9s  |
| Main hero finished loading | 7.4s  |
| Page shift while loading   | 0.19  |
| Blocked button response    | 610ms |

WHAT THE CLIENT RECEIVES

- ✔ Confirm the site pages and services to review.
- ✔ Run Lighthouse, mobile UX, CTA, and trust-signal checks.
- ✔ Fix the highest-impact items first: speed, mobile call flow, form clarity, and trust language.
- ✔ Deliver a before/after report and a plain-English email summary you can actually understand.
- ✔ Recommend ongoing support only when the findings justify it.

RECOMMENDED FIT

A site in this range usually fits the **14-Day Cash-Medical Lead Leak Fix**. If the first audit proves the foundation cannot support the goal, the honest recommendation becomes a conversion rebuild.

Ongoing support is only recommended when there is a clear reason: local visibility, monthly content, monitoring, or post-fix growth work.

NEXT STEP

Start with the free audit if you want this style of plain-English review on your own site.

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